

- C) Polyhydramnios
 - D) Gestational diabetes
- Answer: A) Uterine rupture

69. Ketotic breath in labour indicates:

- A) Hyperglycemia
- B) Dehydration and starvation
- C) Pulmonary edema
- D) Amniotic fluid embolism

Answer: B) Dehydration and starvation

70. Definitive management of obstructed labour with live fetus is:

- A) Symphysiotomy
- B) Destructive operation
- C) Cesarean section
- D) Forceps delivery

Answer: C) Cesarean section

71. Variable decelerations indicate:

- A) Uteroplacental insufficiency
- B) Cord compression
- C) Maternal fever
- D) Fetal anomalies

Answer: B) Cord compression

72. Fetal bradycardia is defined as FHR <:

- A) 100 bpm
- B) 110 bpm
- C) 120 bpm
- D) 130 bpm

Answer: B) 110 bpm

73. In the ABCDE obstetric emergency approach, "D" stands for:

- A) Delivery
- B) Disability
- C) Dehydration
- D) Drugs

Answer: B) Disability

74. Foul-smelling liquor indicates:

- A) Normal labour
- B) Chorioamnionitis
- C) Placenta previa
- D) Fetal wellbeing

Answer: B) Chorioamnionitis

75. Prolonged labour increases risk of:

- A) Fetal macrosomia
- B) Postpartum hemorrhage
- C) Polyhydramnios
- D) Gestational hypertension

Answer: B) Postpartum hemorrhage

76. First step in managing obstructed labour:

- A) Emergency C-section
- B) Call for help and secure IV access
- C) Symphysiotomy
- D) Oxytocin augmentation

Answer: B) Call for help and secure IV access

77. Fetal tachycardia is defined as FHR >:

- A) 140 bpm
- B) 150 bpm
- C) 160 bpm
- D) 170 bpm

Answer: C) 160 bpm

78. Maternal exhaustion in prolonged labour can lead to:

- A) Uterine atony
- B) Placental abruption
- C) Cord prolapse
- D) Shoulder dystocia

Answer: A) Uterine atony

79. Which is NOT a cause of fetal distress?

- A) Uteroplacental insufficiency
- B) Cord compression
- C) Maternal fever
- D) Maternal hypertension without placental involvement

Answer: D) Maternal hypertension without placental involvement

80. If fully dilated with fetal distress:

- A) Emergency C-section
- B) Instrumental delivery
- C) Expectant management
- D) Tocolysis

Answer: B) Instrumental delivery

81. Prolonged active phase is cervical dilation < __ cm/hour:

- A) 1
- B) 1.2
- C) 1.5
- D) 2

Answer: A) 1

82. Concealed placental abruption presents with:

- A) Painless bleeding
- B) Pain, tenderness, no bleeding
- C) Painless, no tenderness
- D) Pain with heavy bleeding

Answer: B) Pain, tenderness, no bleeding

83. In obstructed labour, the mother may have:

- A) Rising pulse and fever
- B) Bradycardia
- C) Hypotension only
- D) No change in vitals

Answer: A) Rising pulse and fever

84. Early decelerations are due to:

- A) Head compression
- B) Cord compression
- C) Uteroplacental insufficiency
- D) Maternal fever

Answer: A) Head compression

85. In fetal distress with meconium, prepare for:

- A) Immediate C-section
- B) Amnioinfusion
- C) Neonatal resuscitation
- D) Tocolysis

Answer: C) Neonatal resuscitation

86. Maternal distress from anxiety can lead to:

- A) Hypotension
- B) Hyperventilation
- C) Bradycardia
- D) Hypoglycemia

Answer: B) Hyperventilation

87. A late sign of obstructed labour is:

- A) Strong contractions

- B) Fetal tachycardia
- C) Foul vaginal discharge
- D) Maternal urge to push

Answer: C) Foul vaginal discharge

88. Fetal distress with reduced variability needs:

- A) Continue monitoring
- B) Oxygen via face mask
- C) Urgent delivery
- D) IV fluids only

Answer: C) Urgent delivery

89. In maternal shock, priority is:

- A) IV fluids and blood
- B) Immediate delivery
- C) Antibiotics
- D) Pain relief

Answer: A) IV fluids and blood

90. The partograph action line indicates:

- A) Normal progress
- B) Need for intervention
- C) Fetal distress
- D) Full dilation

Answer: B) Need for intervention

4. Hyperemesis Gravidarum, APH & Hypertensive Disorders (Q91 – 120)

91. Hyperemesis gravidarum involves weight loss > ___% of pre-pregnancy weight:

- A) 3%
- B) 5%
- C) 7%
- D) 10%

Answer: B) 5%

92. Antepartum hemorrhage is defined as bleeding after ___ weeks gestation:

- A) 20
- B) 24
- C) 28
- D) 32

Answer: C) 28

93. Painless vaginal bleeding in third trimester suggests:

- A) Placental abruption
- B) Placenta previa
- C) Uterine rupture
- D) Vasa previa

Answer: B) Placenta previa

94. Severe pre-eclampsia is defined as BP $\geq$:

- A) 140/90 mmHg
- B) 150/100 mmHg
- C) 160/110 mmHg
- D) 170/120 mmHg

Answer: C) 160/110 mmHg

95. HELLP syndrome includes:

- A) Hemolysis, Elevated Liver enzymes, Low Platelets
- B) Hypertension, Edema, Low Proteinuria
- C) Hemorrhage, Elevated Lactate, Low Perfusion
- D) Hyperglycemia, Elevated Lipids, Low Protein

Answer: A) Hemolysis, Elevated Liver enzymes, Low Platelets

96. Placenta previa Type IV means:

- A) Low-lying placenta
- B) Marginal placenta previa
- C) Partial covering of internal os
- D) Complete covering of internal os

Answer: D) Complete covering of internal os

97. Placental abruption typically presents with:

- A) Painless bleeding
- B) Pain and tenderness
- C) No symptoms
- D) Fetal movements only

Answer: B) Pain and tenderness

98. The drug of choice for preventing eclamptic seizures is:

- A) Phenytoin
- B) Magnesium sulfate
- C) Diazepam
- D) Lorazepam

Answer: B) Magnesium sulfate

99. Hyperemesis can cause deficiency of:

- A) Vitamin C

- B) Vitamin B1 (Thiamine)
- C) Vitamin D
- D) Vitamin K

Answer: B) Vitamin B1 (Thiamine)

100. In suspected placenta previa, vaginal examination is:

- A) Encouraged
- B) Contraindicated
- C) Done routinely
- D) Done only with ultrasound

Answer: B) Contraindicated

101. Eclampsia is defined as:

- A) Hypertension + proteinuria
- B) Hypertension + seizures
- C) Hypertension + edema
- D) Hypertension + headache

Answer: B) Hypertension + seizures

102. First-line antihypertensive in pregnancy is:

- A) Nifedipine
- B) Labetalol
- C) Methyldopa
- D) Hydralazine

Answer: C) Methyldopa

103. A risk factor for placenta previa is:

- A) Primigravida
- B) Previous C-section
- C) Maternal age <20
- D) Normal BMI

Answer: B) Previous C-section

104. Placental abruption can cause:

- A) Disseminated intravascular coagulation
- B) Polycythemia
- C) Hyperglycemia
- D) Hypokalemia

Answer: A) Disseminated intravascular coagulation

105. Gestational hypertension is:

- A) Hypertension before 20 weeks
- B) Hypertension after 20 weeks without proteinuria
- C) Hypertension with proteinuria

D) Hypertension with seizures

Answer: B) Hypertension after 20 weeks without proteinuria

106. Hyperemesis management includes:

A) IV fluids and antiemetics

B) Oral rehydration only

C) Immediate delivery

D) High-fat diet

Answer: A) IV fluids and antiemetics

107. Placenta previa increases risk of:

A) Postpartum hemorrhage

B) Fetal macrosomia

C) Maternal diabetes

D) Polyhydramnios

Answer: A) Postpartum hemorrhage

108. Superimposed pre-eclampsia is:

A) New hypertension in pregnancy

B) Chronic hypertension + new proteinuria after 20 weeks

C) Hypertension with normal labs

D) Hypertension resolved postpartum

Answer: B) Chronic hypertension + new proteinuria after 20 weeks

109. Which is NOT a symptom of severe pre-eclampsia?

A) Headache

B) Visual disturbances

C) Epigastric pain

D) Diarrhea

Answer: D) Diarrhea

110. In APH due to placenta previa at 34 weeks with mild bleeding, management is:

A) Immediate C-section

B) Induction of labour

C) Conservative with observation

D) Discharge home

Answer: C) Conservative with observation

111. Wernicke's encephalopathy in hyperemesis is due to deficiency of:

A) Vitamin C

B) Thiamine

C) Vitamin D

D) Vitamin B12

Answer: B) Thiamine

112. In APH, if vasa previa is suspected:

- A) Wait and watch
- B) Induce labour
- C) Emergency C-section
- D) Steroids only

Answer: C) Emergency C-section

113. Magnesium sulfate toxicity is indicated by:

- A) Loss of patellar reflexes
- B) Hypertension
- C) Tachycardia
- D) Hyperreflexia

Answer: A) Loss of patellar reflexes

114. The most dangerous type of abruption for the fetus is:

- A) Revealed
- B) Concealed
- C) Mixed
- D) Marginal

Answer: B) Concealed

115. In gestational hypertension, delivery is recommended by:

- A) 34 weeks
- B) 37 weeks
- C) 40 weeks
- D) 42 weeks

Answer: B) 37 weeks

116. HELLP syndrome requires:

- A) Bed rest only
- B) Antihypertensives only
- C) Delivery regardless of GA
- D) Magnesium sulfate only

Answer: C) Delivery regardless of GA

117. Chronic hypertension in pregnancy is:

- A) Present before 20 weeks
- B) Develops after 20 weeks
- C) Resolves after delivery
- D) Always with proteinuria

Answer: A) Present before 20 weeks

118. Placental abruption is associated with:

- A) Multiparity only
- B) Trauma and hypertension
- C) Young maternal age
- D) First pregnancy

Answer: B) Trauma and hypertension

119. To confirm placenta previa, use:

- A) Ultrasound
- B) Vaginal examination
- C) CT scan
- D) MRI only

Answer: A) Ultrasound

120. First-line antiemetic in hyperemesis is:

- A) Ondansetron
- B) Metoclopramide
- C) Doxylamine + Pyridoxine
- D) Prochlorperazine

Answer: C) Doxylamine + Pyridoxine

5. Integrated & Complex Clinical Scenarios (Q121 – 130)

121. A woman at 35 weeks with polyhydramnios, diabetes, and fetal macrosomia goes into labour.

The biggest intrapartum risk is:

- A) Cord prolapse
- B) Postpartum hemorrhage
- C) Shoulder dystocia
- D) Uterine rupture

Answer: C) Shoulder dystocia

122. In a patient with severe anemia (Hb 6.5 g/dL) and pre-eclampsia at 32 weeks, priority is:

- A) Immediate delivery
- B) Control BP, transfuse if symptomatic, plan delivery
- C) Iron therapy only
- D) Wait until 37 weeks

Answer: B) Control BP, transfuse if symptomatic, plan delivery

123. A primigravida at 38 weeks with breech, polyhydramnios, and previous C-section should deliver by:

- A) Vaginal breech delivery
- B) External version then vaginal delivery
- C) Elective C-section

D) Induction of labour

Answer: C) Elective C-section

124. In fetal distress with late decelerations and oligohydramnios, the likely cause is:

A) Cord compression

B) Uteroplacental insufficiency

C) Maternal fever

D) Fetal sleep

Answer: B) Uteroplacental insufficiency

125. A woman with APH due to abruption, BP 90/60, tender uterus, no vaginal bleeding. Next step:

A) IV access, fluids, urgent C-section

B) Wait and monitor

C) Give antihypertensives

D) Induce labour

Answer: A) IV access, fluids, urgent C-section

126. Hyperemesis with weight loss 8%, ketonuria, hypokalemia, and confusion. Immediate treatment:

A) IV glucose only

B) IV thiamine, correct electrolytes

C) Anti-emetics only

D) Immediate delivery

Answer: B) IV thiamine, correct electrolytes

127. Placenta previa type II anterior at 36 weeks with mild bleeding. Management:

A) Immediate C-section

B) Admit, steroids, deliver at 37 weeks

C) Send home with follow-up

D) Induce labour now

Answer: B) Admit, steroids, deliver at 37 weeks

128. Prolonged labour with OP, caput +3, no progress in 2nd stage for 3 hours. Next:

A) Continue waiting

B) Oxytocin augmentation

C) Manual rotation or C-section

D) Vacuum without rotation

Answer: C) Manual rotation or C-section

129. HELLP syndrome with platelets 40,000, BP 170/110, 34 weeks. Management:

A) Bed rest and monitoring

B) Antihypertensives only

C) Magnesium sulfate only

D) MgSO₄, antihypertensives, deliver within 24h

Answer: D) MgSO_4 , antihypertensives, deliver within 24h

130. Oligohydramnios + fetal renal agenesis at 28 weeks. Counselling includes:

- A) Excellent prognosis with surgery
- B) Poor prognosis, consider palliative care after birth
- C) Normal delivery at term
- D) Immediate termination

Answer: B) Poor prognosis, consider palliative care after birth